

BEFORE THE BOARD OF PSYCHOLOGY EXAMINERS  
OF THE STATE OF IOWA

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| IN THE MATTER OF<br>THE LICENSE TO PRACTICE<br>AS A PSYCHOLOGIST OF:<br><br>JOHN D. BREWER,<br>Respondent | )<br>)<br>)<br>)<br>)<br>)<br>) | DIA NO. 90DPHPS-1<br><br><br>FINDINGS OF FACT,<br>CONCLUSIONS OF LAW,<br>DECISION AND ORDER |
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On July 20, 1990, Carl Davis, Ph.D., Chairperson of the Iowa Board of Psychology Examiners, filed a Notice of Hearing and Statement of Charges against the Respondent, John D. Brewer. On August 17, 1990, the Respondent filed a Motion for a Continuance. In an Order dated August 22, 1990, the Respondent's motion was granted and the hearing was continued. On October 30, 1990, the Respondent filed a second Motion for a Continuance. The State filed a Resistance to the Motion for a Continuance on October 31, 1990. On October 31, 1990, an Order was issued requiring the Respondent to provide certain documentation. On November 2, 1990, Respondent's attorney filed by FAX a Reply to the state's resistance, and part of the documentation required by the order. On November 7, 1990, Respondent filed by FAX the remainder of the required documentation. On November 21, 1990, an Order was issued which continued the hearing. On December 21, 1990, Respondent's attorney filed an Application for Permission to Withdraw as Counsel and a third Motion for a Continuance. The State filed a Resistance to the Respondent's motion to continue and motion to withdraw. On January 8, 1991, attorneys David S. Wiggins and William J. Wimmer filed an Appearance on behalf of the Respondent. On January 14, 1991, an Order granted Mr. Riccolo's application to withdraw as counsel and continued the hearing to February 15 and 16, 1991. On February 12, 1991, the Respondent filed an Affidavit in support of disqualification. On February 13, 1991, Respondent's attorney filed Exhibits A and B which had been erroneously left off the affidavit. The affidavit, which requested that Board members who had participated in a settlement discussion held on February 8, 1991 be disqualified, was denied at the beginning of the hearing on February 15, 1991.

The hearing for the above case was held on February 15 and 16, 1991 in the first floor side two conference room, Lucas State Office Building, Des Moines, Iowa. Upon the Respondent's request, the hearing was closed to the public pursuant to Iowa Code section 258A.6(1) and 645 IAC 240.207. The Respondent, John D. Brewer, was present and was represented by his attorneys, David S. Wiggins and William J. Wimmer. However, William J. Wimmer did not actually appear at the hearing itself. Chris Odell, Assistant Attorney General, appeared on behalf of the State. The Board members who began participation in the hearing were: Rick L. Jennings, psychologist; Cynthia Lyon, public

member; Kathryn Gerken, psychologist; Carl Davis, psychologist and Board chairperson; and Phyllis Hansell, psychologist. During the hearing, Carl Davis recused himself and did not participate further in the hearing or in the decisionmaking of the Board. A court reporter recorded the proceedings. The deliberations were tape recorded. The undersigned Administrative Law Judge from the Iowa Department of Inspections and Appeals presided.

After hearing the testimony, the Board convened in closed executive session pursuant to Iowa Code section 21.5(1)f to discuss the decision to be rendered in this contested case. The Board adjourned and directed the undersigned to prepare this Board decision. This findings of fact, conclusions of law, and decision and order of the Board is a unanimous decision.

On February 18, 1991, the Respondent filed a second Motion to Disqualify which reflected an oral motion made at the hearing and denied.

#### THE RECORD

The evidentiary record in this case includes the above notice, motions, orders, application, letter, resistances, appearance, the recorded testimony of the witnesses, Respondent's Exhibits A through O and State's Exhibits 1 through 4 and 6 through 8.

#### FINDINGS OF FACT

1. The Respondent, John Douglas Brewer, is a psychologist licensed to practice in Iowa. (Respondent's Exhibit L).
2. In June of 1987, the patient presented to Dr. Brewer for an intake interview stating that her problems were extreme nervousness and agoraphobia. She began weekly visits with Dr. Brewer and paid in full in cash at the time of each visit. She stated that she hoped to gain self-esteem and an ability to act competently on her own behalf. Dr. Brewer saw this patient every week throughout the course of therapy. (testimony of Dr. Brewer, the patient; State's Exhibit 2).
3. At the time she began seeing Dr. Brewer professionally, the patient was a 33-year-old single female who had a long-term volatile relationship with a boyfriend. Her parents were deceased. She had a younger sister with whom she was close, a close friend in California, a close relationship with two aunts who resided in her community. The patient did not work outside the home. She was described as very intelligent and as having an extensive vocabulary. (testimony of the patient, Dr. Brewer; State's Exhibit 2).
4. When this patient came to Dr. Brewer, she was taking Ativan and appeared to Dr. Brewer to be concerned about the possibility that her Ativan would run out. She requested that Dr. Brewer work with her physician to make sure that she continued to

receive Ativan. Dr. Brewer did this, and for a time, contacted this patient's physician to request a refill of the Ativan prescription. Eventually, by mutual agreement, this patient began requesting the Ativan directly from her doctor without Dr. Brewer's intervention or assistance. The patient did not appear to Dr. Brewer to be dependent on Ativan. (testimony of Dr. Brewer, the patient; State's Exhibit 2).

5. This patient had at times experienced panic attacks and suffered from depression since puberty. She had approximately seven years of therapy prior to coming to Dr. Brewer and initially told him of her previous therapy in Arizona. Dr. Brewer at no time requested her prior psychiatric records. (testimony of Dr. Brewer, the patient).

6. In September of 1987, after seeing this patient for 12 sessions, Dr. Brewer diagnosed her as having "Axis I, Agoraphobia with Panic Attacks although the panic attacks had been much rarer; Axis II, Some features of Avoidance Personality, Dependent Personality, and intense fear of rejection or ridicule; Axis III, None Noted; Axis IV, Minimal; Axis V, Fair." His stated treatment goals were to "1) Encourage increased self-confidence through creativity and intellect; 2) Examine relationship with boyfriend; 3) Encourage assertive and less dependent behavior." This patient expressed anxiety, shyness, anger, and an intense fear of rejection. (testimony of Dr. Brewer; State's Exhibit 2).

7. Both the patient and Dr. Brewer testified that she felt somewhat better as a result of Dr. Brewer's treatment from the period June of 1987 through July of 1989. However, it does not appear that this patient made significant progress from Dr. Brewer's treatment. Dr. Brewer described this patient's progress as meager and with relapses. The patient told Dr. Brewer that at times she drank excessively and in the morning. However, she did not feel that she was an alcoholic, nor did Dr. Brewer. (testimony of Dr. Brewer, the patient; State's Exhibit 2; Respondent's Exhibit Q).

8. In January of 1989, the patient's boyfriend moved out of town to attend school. She continued the relationship on a long-distance basis. (testimony of the patient).

9. Throughout the course of therapy, it did not appear that Dr. Brewer was cognizant of goals for this patient. The two talked about whatever subject came up without direction on the part of the therapist. In April of 1989, the patient asked Dr. Brewer if he had other patients who found him attractive. The two discussed the patient's transference as well as issues relating to ethics and psychological practice. Dr. Brewer testified that the two discussed that a psychologist could not see a patient socially while she was his patient, but that this was often circumvented by the termination of the therapist-patient relationship and immediate formation of a social relationship. In the spring of 1989, Dr. Brewer and the patient perused a

Victoria's Secret catalog including the lingerie section during a session. Dr. Brewer indicated that this was a happenstance occurrence. Looking at the Victoria's Secret catalog was completely inappropriate due to a clear risk of misinterpretation by this patient. (testimony of Dr. Brewer, the patient; State's Exhibit 2; Respondent's Exhibit O).

10. Dr. Brewer's intake notes appear to be complete. However, his progress notes are extremely brief. Dr. Brewer testified that he did not keep long progress notes because he did not want to do anything that was potentially harmful to a patient. The Board is concerned that the notes have minimal clinical usefulness. At no point in his progress notes did Dr. Brewer discuss looking at the Victoria's Secret catalog with the patient, nor did he discuss his reasons for doing so or the ostensible therapeutic benefit. (testimony of Dr. Brewer; State's Exhibit 2; Respondent's Exhibit O).

11. During the sessions Dr. Brewer told jokes of a sexual nature and he and the patient engaged in suggestive banter. (testimony of the patient, Dr. Brewer, patient's sister).

12. Dr. Brewer and his wife separated at the beginning of July 1989. Dr. Brewer attended therapy on his own behalf to discuss his marital problems. He increasingly began to talk about his own marital problems during the sessions with the patient. In the summer of 1989, the two began discussing Dr. Brewer's problems with his marriage approximately half of the therapy time. This was entirely inappropriate. Dr. Brewer agreed at the hearing that the sessions were as much for his benefit as they were for the patient with the patient serving as his "sounding board." However, the patient continued to pay for the visits as if they were therapy for her. (testimony of Dr. Brewer, the patient, patient's sister; State's Exhibits 1, 2; Respondent's Exhibit O).

13. Dr. Brewer extensively self-disclosed subjects of an extremely personal nature with this patient during therapy. The self-disclosure was too extensive in detail and too personal in nature to have provided any benefit to this patient. There was no therapeutic justification provided for the extent or quality of Dr. Brewer's self-disclosure. (testimony of Dr. Brewer, Dr. Rogers, the patient, patient's sister; State's Exhibits 2, 6, 7; Respondent's Exhibits F, G, H, I, O).

14. Dr. Brewer agreed that the two had discussed the patient's transference as early as April of 1989. It appeared throughout the spring and summer of 1989 that the patient was experiencing a high degree of transference. Although Dr. Brewer testified that this was subtle, all other evidence presented indicated that it was blatant. Dr. Brewer testified that he was not aware of the degree of this patient's transference. The Board does not find Dr. Brewer's testimony to be credible. It is the responsibility of the competent professional to manage transference in a

professional and therapeutic manner. There was no indication whatsoever that this was done by Dr. Brewer. Not only was this not done, but Dr. Brewer used the increasingly personal and friendly therapist/patient relationship for his own benefit. Although he did not so testify at the hearing, it is obvious that Dr. Brewer experienced an emotional response to this patient which negatively impacted treatment. In the summer of 1989 during a therapy session, Dr. Brewer said to the patient that it would be fun to visit each other's homes and drink wine in front of each other's fireplaces. Dr. Brewer called this patient "Liebchen" (which means "beloved") in sessions. (testimony of Dr. Brewer, the patient, patient's sister; State's Exhibit 2; Respondent's Exhibit O).

15. At some point during the therapy, Dr. Brewer showed this patient scars which he has on both his shoulder and his abdomen. Such behavior was beyond the bounds of reasonable and appropriate conduct. Dr. Brewer could not have modestly and professionally revealed these scars to this patient and there would have been no therapeutic benefit to have done so. (testimony of the patient, Dr. Brewer, Dr. Rogers; State's Exhibits 8a-c).

16. At one point in the therapy the patient critically noted the varying shades of white in Dr. Brewer's outer clothing. Dr. Brewer began unbuttoning his shirt to show the patient the color of his undershirt and asked if that was better. The two engaged in sexually suggestive banter at that point as well as during other sessions. (testimony of the patient, Dr. Brewer).

17. On July 20, 1989, the patient's sister accompanied her to a therapy session. During the session, Dr. Brewer invited the patient and her sister to go to dinner with him at a restaurant. The sister declined. During the session, Dr. Brewer, the patient, and the patient's sister made arrangements for the patient and her sister to visit and tour Dr. Brewer's home. After this July 20th visit, the patient's sister informed the patient that she believed that Dr. Brewer was personally interested in the patient, wanted to date her, and that she should stop seeing him professionally. The patient's sister expressed discomfort with the amount of therapy time spent discussing Dr. Brewer's problems.

A number of the discussions in therapy concerned Dr. Brewer's unusual victorian house and the work that he was doing on it. The unusual nature of Dr. Brewer's house is irrelevant to the issues before this Board. There was no therapeutic benefit to the patient for the invitation to dinner, the invitation to Dr. Brewer's home, or for the discussions of his house to the extent they occurred. To the contrary, these actions put the patient at risk. Dr. Brewer's patient notes do not reflect either the invitation or the visit to his home. (testimony of Dr. Brewer, the patient, patient's sister, Dr. Rogers, Dr. Enzl, Ms. Blake; State's Exhibit 2; Respondent's Exhibit O).

18. On the evening of July 21, 1989, the patient and her sister visited Dr. Brewer's home for about 45 minutes. Dr. Brewer and the patient each had a glass of wine, and Dr. Brewer then gave the patient and her sister a tour of his home including the third floor bedroom area. As they were leaving, the patient said to Dr. Brewer, "I'll see you next on a paying basis." Dr. Brewer responded, "Not necessarily." (testimony of Dr. Brewer, the patient, patient's sister, Dr. Enzl, Dr. Rogers, State's Exhibit 3; Respondent's Exhibits A, B).

19. The patient's sister returned to New York in between the July 21st visit to Dr. Brewer's home and the patient's July 27th therapy visit. During the July 27, 1989 visit, the patient asked Dr. Brewer if the invitation to go to the restaurant was still good for her alone. While Dr. Brewer initially responded with some hesitancy, he eventually agreed. The patient then stated that she was menstruating. Dr. Brewer responded jokingly. The patient testified that it was clear to her from the conversation and discussion about the restaurant that the outing would include a sexual encounter. Dr. Brewer testified that he was unaware of the strength of the patient's transference, and he was unaware of the patient's feeling that this would be a sexual encounter. This testimony is not credible, particularly given the remark about the patient's menstruation. In any event, Dr. Brewer never attempted, as a competent professional would have, to clarify this statement within a professional context. It is the Board's belief that this was a social occasion. Dr. Brewer did nothing to indicate to this patient that she should interpret this outing as anything other than a date. Dr. Brewer's progress note for July 27, 1989 contains nothing regarding this significant discussion. It was absolutely improper for Dr. Brewer to take this patient out on a date. (testimony of Dr. Brewer, the patient, Dr. Rogers, Dr. Enzl; State's Exhibits 2, 8a-c; Respondent's Exhibits A, B, O).

20. In his testimony, Dr. Brewer first described this outing as a "field trip." When Dr. Brewer was interviewed about this evening by the Board investigator in March of 1990, he justified it as being a field trip therapeutic to the patient's agoraphobia. Also during the investigation interview, Dr. Brewer testified that he discussed with the patient rules and goals for the outing. In his testimony at the hearing, Dr. Brewer did not state anything about any discussion of such rules or goals, and there was no mention of this outing in the progress notes. At the hearing Dr. Brewer agreed that this outing was not in fact a field trip, that there was no related therapeutic benefit, and that the patient had already been to this restaurant with her boyfriend. This outing was particularly inappropriate given this patient's diagnosis, the way the therapy had progressed, and Dr. Brewer's own counter-transference. Based on the testimony of both Dr. Brewer and the patient, this trip was a date. (testimony of the patient, patient's sister, Dr. Brewer, Dr. Rogers, Dr. Enzl; State's Exhibits 2, 8a-8c; Respondent's Exhibits A, B, O).

21. Dr. Brewer picked up the patient at her home in his sports car at approximately 7:30 p.m. on July 28, 1989 and the two proceeded to the restaurant. Dr. Brewer and the patient each had a meal and one marguerita and then split either one or two more margueritas. Dr. Brewer knew that the patient was taking Ativan at this time. One of the effects of Ativan is an increased sensitivity to alcohol. The two engaged in banter of a suggestive nature during dinner. Dr. Brewer paid for the dinner and drinks and then took the patient to a bar where each had one beer. Here the two met and conversed with a friend of Dr. Brewer's. Dr. Brewer then took the patient to a second bar. Dr. Brewer then took the patient to his home, arriving at approximately 10:30 p.m. Dr. Brewer testified that he took the patient to his third floor bedroom to show her the work he had done on the room since she had been there the week before. This testimony is not credible. Dr. Brewer denied that any sexual activity occurred at this time and place. The patient testified to clear memories of sexual contact and to a morning after observation regarding her personal hygiene that cause the Board to believe that some degree of sexual encounter took place in Dr. Brewer's bedroom that night. Although the Board believes it is probable that actual sexual intercourse occurred, it is not willing to make that finding of fact. While the Board finds the patient's testimony far more credible than Dr. Brewer's, she acknowledges some lack of clarity in recall, attributing it to her alcohol ingestion. Dr. Brewer's version of the events reveals substantial improprieties on his part. His explanations of those improprieties are not believable. It does not appear that his motive and intentions were professional with respect to this patient.

Dr. Brewer's failure to extricate himself from this situation despite multiple opportunities to do so affects his credibility. His explanation of his supposed inability to get this patient out of his bedroom after he had gotten her into his bedroom is not believable. It is clear that some degree of physical sexual encounter occurred in Dr. Brewer's bedroom on July 28, 1989. It is irrelevant whether actual intercourse occurred since there were such multiple, gross violations of professional ethics and competence in this incident and case. Dr. Brewer's testimony regarding the specifics of being in the bedroom with the patient and then leaving is not credible. Dr. Brewer's testimony as to his behavior and the occurrences of that night and the next morning are conflicting to the degree that the Board does not find his testimony credible. (testimony of Dr. Brewer, the patient, Dr. Enzl, Dr. Rogers; State's Exhibits 2, 4, 8a-8c; Respondent's Exhibits B, A, O).

22. At no time when contemplating or planning the evening did Dr. Brewer consider or prepare for the protection of the confidentiality of the patient-therapist relationship, the very existence of which he is obligated not to disclose. Dr. Brewer was questioned in the hearing regarding what provisions he had made

should he and the patient be seen by someone that he knew and asked about their relationship. Dr. Brewer testified that he had not considered this an issue. Clearly, he had not professionally or competently considered the requirements of patient confidentiality when he took his patient to the restaurant and the two bars. (testimony of Dr. Brewer).

23. Dr. Brewer testified that he realized how much trouble he was in once he was in the bedroom on the third floor on the evening of July 28, 1989. He testified that he then went downstairs, and although he thought about calling a friend to come and be in the home with him, he did not do so. He also testified that he felt that the situation could not get any worse, and so therefore he did not ask anyone to come to his home. Even according to Dr. Brewer's own testimony, he never strongly and clearly told the patient to leave his bedroom. Dr. Brewer should have realized that he was in an untenable position as a professional and there were alternatives he could have used to limit the breach of confidentiality and still conduct himself in a professional manner. He could at that point have requested the assistance of a colleague. This is further reason the Board does not believe Dr. Brewer's testimony regarding what happened in the bedroom. In any event, there is no dispute that the patient stayed overnight in Dr. Brewer's home on the night of July 28, 1989. The next morning, there is also no dispute that the patient asked Dr. Brewer to make love. Dr. Brewer testified that he declined and then took the patient home. The patient invited him to tour her house and he accepted and participated in a tour of her house. During the tour the patient again asked Dr. Brewer if he would like to make love but he declined. When Dr. Brewer left the patient's home in the morning she asked him if she should keep her next appointment and he said she should. (testimony of the patient, Dr. Brewer, Dr. Rogers; State's Exhibits 8a-8c).

24. The patient's next appointment was on August 3, 1989. Dr. Brewer's progress notes for this date do not contain any mention of the date of July 28/29, 1989. (testimony of the patient, Dr. Brewer; State's Exhibit 2; Respondent's Exhibit O).

25. Dr. Brewer was seeing his own private therapist at the time regarding his marital problems but stated that he did not discuss the date with his therapist. Two of the reasons the Board believes there is no question that this was an improper relationship are the lack of documentation of the event in patient progress notes, and Dr. Brewer's failure to discuss the event with his own therapist. (testimony of Dr. Brewer; State's Exhibit 2; Respondent's Exhibit O).

26. The patient continued to see Dr. Brewer on a professional and weekly basis from August through December of 1989 and continued to pay for the sessions. Dr. Brewer testified that one of his primary reasons for continuing to see the patient was to try to remedy the problems he had created. The patient testified therapy was continued so the personal relationship could

continue. The Board believes that the therapeutic benefit for this patient had ended and that the patient should have been referred elsewhere. (testimony of Dr. Brewer, the patient; State's Exhibits 1, 2; Respondent's Exhibit O).

27. The patient never testified that this was a case of her seduction by Dr. Brewer. She testified that she was in love with Dr. Brewer, wanted sexual activity to take place, and she actively pursued such. This does not excuse Dr. Brewer's behavior. It was his responsibility to professionally and effectively manage the patient's transference, and his counter-transference. Dr. Brewer did not do so. A large amount of the discussion in therapy from August through December of 1989 was that of the relationship between Dr. Brewer and this patient with the patient wanting to continue her personal relationship with Dr. Brewer. Dr. Brewer's notes do not mention the problem which he had created, nor anything regarding how he professionally dealt with the situation. There is no mention of transference, although this was clearly being discussed at length. The patient's last professional visit with Dr. Brewer occurred on December 14, 1989. At no point from August of 1989 through December of 1989 did Dr. Brewer recommend or attempt to transfer this patient's care to another psychologist. (testimony of the patient, Dr. Brewer, Dr. Enzl, Dr. Rogers; State's Exhibits 2, 8a-8c; Respondent's Exhibit O).

28. In December of 1989, the patient called Dr. Brewer at his home and asked if she and her sister could visit him there. Dr. Brewer so permitted. The patient and her sister briefly visited Dr. Brewer in his home where a male friend and Dr. Brewer's children were also present. Dr. Brewer does not mention this visit in his patient progress notes. (testimony of Dr. Brewer, the patient, patient's sister; State's Exhibit 2; Respondent's Exhibit O).

29. In late December of 1989, the patient's boyfriend returned to her home and discovered the existence of the relationship between Dr. Brewer and the patient. His reaction was strongly negative and he wrote threatening letters to Dr. Brewer, unbeknownst to the patient. Dr. Brewer wrote a professional letter to the boyfriend stating that he would be happy to discuss the boyfriend's concerns with him. However, the boyfriend refused to see Dr. Brewer or talk to him and pushed this patient to file a complaint with the Board. The patient found out that her boyfriend and Dr. Brewer had corresponded and then wrote to Dr. Brewer. (testimony of Dr. Brewer, the patient; Respondent's Exhibits C, D, E, F, G; State's Exhibit 6).

30. After having received threatening correspondence from the patient's boyfriend and not knowing whether the patient was aware or unaware of the correspondence from the boyfriend, Dr. Brewer decided to attempt to transfer this patient to another therapist. The patient wrote a letter to Dr. Brewer. Dr. Brewer then wrote a letter to the patient which cannot be characterized

as professional according to any standards. The handwritten letter which was not on his letterhead was personally hand delivered by Dr. Brewer. (testimony of Dr. Brewer, the patient; State's Exhibits 5, 7; Respondent's Exhibits F, M).

31. The patient called Dr. Brewer, and the two arranged to meet "accidentally" at a drug store, which they did and then went to a donut shop to discuss the situation. At that meeting the patient told Dr. Brewer that she was seeing another therapist. Dr. Brewer told the patient that if a complaint was filed with the Board, he would admit to a dual relationship, but he would never admit that a sexual liaison had taken place. The patient had been seeing another therapist since January 1990. In January of 1990, the patient still expressed love for Dr. Brewer and since that time has been depressed. As of the date of the hearing, the beginning of 1991, she is less depressed but remains confused. It appears that this patient, even as late as January of 1990, was interested in pursuing a personal relationship with Dr. Brewer. The cause of the patient's confusion and depression was the personal relationship with Dr. Brewer and its termination. (testimony of Dr. Enzl, the patient, Dr. Brewer, Dr. Rogers; State's Exhibit 6; Respondent's Exhibits F, G, H, I).

32. There was no evidence presented at the hearing that Dr. Brewer behaved similarly toward any other patients. Additional testimony indicated that Dr. Brewer acted professionally with other patients, and has support in his community. However, the transference in this case was so blatant and so obvious that if, as he testified, Dr. Brewer did not recognize it in this patient, he would not recognize it in other patients where it would be more subtle. The Board also finds that the evidence at the hearing demonstrated that Dr. Brewer did not competently and professionally treat the patient's transference, nor did he professionally and competently deal with his own counter-transference. (testimony of Dr. Brewer, the patient, Dr. Enzl, Dr. Rogers, Mr. Zevenbergen; State's Exhibit 2; Respondent's Exhibits J, K, O).

33. At some point, another patient gave the Respondent a hand-rolled cigarette. The Respondent gave this to the patient in this case. Although the patient testified that she went home and smoked the cigarette with her boyfriend and it was marijuana, the evidence is inadequate to establish that in fact Dr. Brewer gave marijuana to the patient. (testimony of Dr. Brewer, the patient; Respondent's Exhibit N).

34. Dr. Brewer's personal interest in this patient, i.e., his counter-transference, impaired his professional judgment with this patient. (testimony of Dr. Brewer, the patient, patient's sister, Dr. Enzl, Dr. Rogers; State's Exhibits 2, 7; Respondent's Exhibit O).

35. The patient was exploited by Dr. Brewer. She was used as a listening post for him, and provided emotional and personal

support for Dr. Brewer in the therapy sessions. (testimony of Dr. Brewer, the patient).

36. There is overwhelming evidence in this record of the patient's and Dr. Brewer's personal interest in each other. Entirely apart from whether any actual sexual encounter took place, this psychologist's treatment of this patient was unprofessional. It was Dr. Brewer's responsibility as the professional psychologist to handle this mutual transference and counter-transference in a competent and professional manner. He makes his living interpreting people's behavior; it is not credible that he did not see the overwhelming indications of this patient's interest in him. It was Dr. Brewer's obligation as a professional psychologist to react in a competent and professional manner in dealing with any manipulation or seductive behavior on the part of this patient. The Board simply does not believe that Dr. Brewer was unaware of the extent of this patient's very intense feelings for him. The Board believes that he was aware of it and he was meeting his own needs. The patient's visits became as enjoyable to Dr. Brewer as they were to the patient. Dr. Brewer's attraction to this patient and his inability to professionally handle his own attraction clouded his judgment. This sort of reaction is to be anticipated and effectively managed. Dr. Brewer not only did not do this, his behavior encouraged her feelings. Dr. Brewer called this patient by a pet name, "Liebchen", in therapy. Dr. Brewer gave this patient hugs at the end of his therapy sessions. In a patient with this diagnosis, who had already discussed her transference as early as April, any physical contact was contraindicated. In a patient with this diagnosis, it is incumbent upon the treating psychologist to make the necessary professional and personal boundaries very clear and direct. Evidence indicates that this was not done. (testimony of the patient, Dr. Brewer, patient's sister, Dr. Enzl, Dr. Rogers; State's Exhibits 6, 7; Respondent's Exhibits A, B, F-J, O).

37. The patient was a credible witness. She appeared to be truthful and candid with the Board. Her admission that she was interested in Dr. Brewer, wanted to be sexually involved with him, and did not claim that Dr. Brewer had seduced her enhanced her credibility. This patient testified she wanted both a personal relationship with Dr. Brewer and a sexual liaison with him on July 28, 1989. Although the patient did not provide the same amount of detail each time she discussed what had happened, her testimony was not in conflict with itself. It appeared that at some points this patient provided additional detail to a story that she had already given. Given this patient's alcohol and Ativan use, her lack of sequential memory of all events of July 28/29, 1990 was not surprising.

The patient shared the details of the July 28th date with her sister and her friend, Mary, soon after the date occurred. This adds to her credibility.

The patient sought further counseling from Dr. Enzl after she terminated her therapy with Dr. Brewer. She told this therapist about her relationship with Dr. Brewer, including the July 28th date. Dr. Enzl believes the patient is telling the truth. (testimony of the patient, patient's sister, Dr. Enzl, Dr. Brewer; State's Exhibit 2, Respondent's Exhibit O).

38. Even after having had the experience of this relationship, when asked at the hearing about his understanding of the parameters of dual relationships, Dr. Brewer did not demonstrate any real understanding of such rules. Therefore the Board has serious fear that Dr. Brewer will violate these rules. Contrary to Dr. Brewer's testimony, the applicability of rules regarding dual relationships does not depend upon the particular patient. Dr. Brewer's other patients appear to be in potential jeopardy because of his demonstrated lack of understanding and his behavior in this case as evidenced by his own testimony. (testimony of Dr. Brewer).

39. In a number of places Dr. Brewer's testimony was in conflict with itself. Dr. Brewer stated at least four different versions of what happened once the two were in the bedroom. The changes he made on critical points contributed to the Board finding his testimony not credible. His testimony conflicted with his own personal letter written to this patient and also with the statements he made to Dr. Rogers in the March 1990 interview. There were a number of places where ethical "red flags" should have gone up for Dr. Brewer, but he did nothing to extricate himself. Even if his own testimony is to be believed, it was not until the two were actually in his bedroom that he recognized his ethical dilemma. According to his testimony, after he realized he was in such a precarious position, he did nothing other than to suggest his and his patient's departure from his home. Dr. Brewer's explanation of his supposed inability to get this patient out of his bedroom once they were in his bedroom is simply not credible. His own patient progress notes reveal nothing of what was actually happening in and out of therapy in a number of significant respects. While some of the minor discrepancies can be attributed to inability to recall details of events which had occurred some time before, many of the discrepancies were major factual items. Dr. Brewer's version of the events contains substantial improprieties and his explanations of those improprieties are not believable. It does not appear that his motive and intentions were professional with respect to this patient. (testimony of Dr. Brewer, Dr. Rogers; State's Exhibits 2, 7, 8a-8c; Respondent's Exhibit O).

40. Dr. Brewer and this patient's relationship became increasingly informal, evolving into a friendship rather than a professional relationship. The sessions became conversations between friends on unplanned topics. No goals were stated and no direction was provided by Dr. Brewer. There were a number of points at which warnings of potential ethical breaches should have become readily apparent to Dr. Brewer and it is

incomprehensible to this Board that this did not occur, or that Dr. Brewer ignored these. The Board believes that Dr. Brewer knew what was happening and proceeded anyway. Although the Board believes that Dr. Brewer was generally unaware of the parameters of his professional code, there are some violations that are so blatant that the Board believes that Dr. Brewer chose not to abide by the professional code of ethics in this case. Dr. Brewer demonstrated insufficient ethical restraint. This is a serious reflection on his character. His description of the evening as a field trip appears to be a reconstruction and an attempt to retroactively justify his behavior. (testimony of Dr. Brewer, the patient, patient's sister, Dr. Enzl, Dr. Rogers; State's Exhibits 8a-8c).

41. The patient had sexual fantasies regarding Dr. Brewer for approximately a week prior to July 28, 1989. Although the Respondent's attorney attempted to show that the patient could have confused these fantasies with a personally believable recollection of an actual sexual experience, this is not a credible explanation. (testimony of the patient).

42. Dr. Brewer showed very poor judgment in purchasing alcohol for a patient, particularly one who was taking Ativan. This is but one of many poor judgments made by Dr. Brewer both in and outside of his office. (testimony of Dr. Brewer, the patient; State's Exhibit 4).

43. According to Dr. Enzl, Dr. Brewer's relationship with the patient caused increased depression due to her disappointment and confusion over the relationship. Although her ongoing relationship with her boyfriend may well have been a factor in her psychological distress, this patient's depression was exacerbated by her experience with Dr. Brewer. He provided her with inadequate treatment, and used her for his own personal benefit. This made future resolution of the patient's problem much more difficult. (testimony of patient number one, Dr. Brewer, Dr. Enzl; State's Exhibit 2, Respondent's Exhibit O).

44. Evidence was presented that there was frequent sexual innuendo in therapy sessions, during the social contacts outside of the office, and in the letter Dr. Brewer hand delivered to his patient. Dr. Brewer's remarks and his management of his patient's remarks present a serious negative reflection on his professional judgment. (testimony of Dr. Brewer, Dr. Enzl, the patient, patient's sister, State's Exhibit 7).

45. Regardless of whether or not sexual behavior had taken place on July 28/29, 1989, Dr. Brewer should have terminated his professional relationship with this patient at the session on August 3, 1989. In addition, Dr. Brewer should have considered terminating the professional relationship much earlier. In early April, this patient revealed her transference. Once it became apparent that Dr. Brewer was experiencing problematic counter-transference, and that he could not treat his patient in a

professional manner, he should have terminated their professional relationship. It was his responsibility to therapeutically manage this relationship. Dr. Brewer admitted at the hearing that the patient's visits were as much for his benefit as for hers with the patient serving as a sympathetic listener during the sessions. Once his personal benefit from the relationship preempted his patient's benefit, he should have sought professional consultation and ethically transferred the case. (testimony of Dr. Brewer, the patient, patient's sister, State's Exhibit 2, Respondent's Exhibit O).

#### CONCLUSIONS OF LAW

1. 645 Iowa Administrative Code section 240.212(1)"c" provides that:

The Board may revoke or suspend a license, or place a license on probation for any of the following reasons: 240.212(1) All grounds listed in Iowa Code section 147.33 which are: . . . c. Knowingly making misleading, deceptive, untrue or fraudulent representations in the practice of a profession or engaging in unethical conduct or practice harmful or detrimental to the public. Proof of actual injury need not be established.

The Respondent violated this rule. The Respondent's practice both inside the therapy sessions and outside the therapy sessions was harmful to this patient. The Respondent engaged in unethical conduct and practice harmful and detrimental to the public and to this patient in particular.

2. In addition, the Board may revoke or suspend for the following violation:

645 IAC 240.212(6) Negligence by the licensee in the practice of the profession, which is a failure to exercise due care including negligent delegation to or supervision of employees or other individuals, whether or not injury results; or any conduct, practice or conditions which impair the ability to safely and skillfully practice the profession.

The Respondent violated this rule. He was negligent in the practice of his profession and failed to exercise due care with this patient. His conduct impaired his ability to safely and skillfully treat the patient. His personal problems affected his ability to professionally deal with this patient but do not in any way excuse his behavior. The Respondent's personal feelings impaired his ability to exercise sound professional judgment.

3. The Board further may revoke or suspend a license or place a licensee on probation for a violation of 645 IAC 240.212(7): "Prohibited acts consisting of the following: . . . f. Any sexual intimidation or sexual relationship between a psychologist and a

client." The Respondent violated this rule by having a sexual relationship with the patient while she was in his professional care.

4. The Board may further discipline licensees for failure to comply with the following rules of ethical conduct and practice. 645 Iowa Administrative Code section 240.212(14)"a"(5) provides:

As practitioners, psychologists know that they bear a heavy social responsibility because their recommendations and professional actions may alter the lives of others. They are alert to personal, social, organizational, financial or political situations or pressures that might lead to misuse of their influence.

The Respondent violated this rule. His unprofessional actions detrimentally altered the life of his patient. He was not alert to personal situations or pressures which led to misuse of his influence.

5. 645 Iowa Administrative Code section 240.212(14) further states the Board may discipline for failure to comply with the following rule of ethical conduct and practice, specifically:

b. Competence.

(5) Psychologists recognize that their effectiveness depends in part upon their ability to maintain effective interpersonal relations, and that aberrations on their part may interfere with their abilities. They refrain from undertaking any activity in which their personal problems are likely to lead to inadequate professional services or harm to a client; or, if engaged in such activity when they become aware of their personal problems, they seek competent professional assistance to determine whether they should suspend, terminate or limit the scope of their professional or scientific activities.

The Respondent seriously violated this ethical rule. He provided inadequate services and harmed his patient due to his personal problems, needs and unmanaged feelings for this patient. Violations of this ethical rule to the extent they occurred resulted in Dr. Brewer's incompetent treatment of this patient. There was evidence that Dr. Brewer sought professional assistance to resolve his marital problems. However, he did not seek professional assistance with regard to this patient, his relationship with her, or his treatment of her.

6. The Respondent violated 645 Iowa Administrative Code section 240.212(14)"f"(1) which states:

Psychologists are continually cognizant of their own needs and of their inherently powerful positions vis-a-vis clients, in order to avoid exploiting their trust and dependency. Psychologists make every effort to avoid dual relationships with clients or relationships which might impair their professional judgment or increase the risk of client exploitation. Examples of such dual relationships include treating employees, supervisors, close friends or relatives. Sexual intimacies with clients are unethical.

The Respondent should have been cognizant of his own needs and of his own inherently powerful position vis-a-vis this patient. He made no apparent effort to avoid a dual relationship and in fact actively participated in creating such a relationship. His professional judgment was impaired and he exploited this patient. He was sexually intimate with this patient and thus behaved unethically.

7. The Respondent violated 645 Iowa Administrative Code section 240.212(14)"f"(5) which states "the psychologist attempts to terminate a clinical or consulting relationship when it is reasonably clear that the consumer is not benefiting from it." By his own testimony, Dr. Brewer agreed that the continuing relationship for which this patient was paying him and which was supposed to be a psychologist-patient relationship, clearly became as beneficial to him as it was to the patient. Their relationship became a friendship rather than a psychologist-patient relationship and was of limited professional benefit to the patient. The relationship became sexual. Dr. Brewer did not attempt to terminate the relationship when it should have been clear to him to do so.

8. 645 Iowa Administrative Code section 240.212(7)"d" provides that a prohibited act is "obtaining, possessing, or attempting to obtain or possess a controlled substance without lawful authority; or selling, prescribing, giving away, or administering controlled substances." The Respondent is not guilty of a violation of this rule.

9. Iowa Code section 147.55 (1989) states that

A license to practice a profession shall be revoked or suspended when the licensee is guilty of any of the following acts or offenses: . . . 3. Knowingly making misleading, deceptive, untrue or fraudulent representations in the practice of a profession or engaging in unethical conduct or practice harmful or detrimental to the public. Proof of actual injury need not be established.

The Respondent is guilty of a violation of Iowa Code section 147.55(3) (1989). He engaged in unethical conduct, and he engaged in practice which was harmful and detrimental to the patient, a member of the public.

10. The Respondent admitted a violation of the charged Code sections, except for 645 Iowa Administrative Code section 240.212(7)"d". He did not admit that sexual intercourse took place on July 28, 1989 with the patient. Dr. Brewer is not guilty of sexual intimidation.

#### DECISION AND ORDER

The Board is aware after hearing testimony that this case involves complex issues of a broad nature. The Respondent's license is not being revoked only because there is no proof that anything similar has ever occurred with another patient. However, the Respondent's behavior with this patient involves multiple and serious violations of Board rules, statutes, professional ethics, and standards of competence. Dr. Brewer betrayed the public trust in professionals and this patient's trust in him. Given this Respondent's actions in this case, the Board is unable to trust him with other patients, particularly those with problems similar to this patient's. If the Respondent could not make effective decisions in this case, it is unlikely that he will be able to make such decisions in similar cases. The gravity of the offense with respect to this patient requires that he serve a lengthy suspension. Dr. Brewer has demonstrated both ethical and technical deficiency in his management of his psychotherapeutic relationship with this patient. He has demonstrated a serious lack of judgment, and the Board has no assurance that by his mere partial re-education, he will have adequately benefited to ensure his future technical competence and ethical awareness in treating this type of a patient. A further appearance before the Board after re-education is therefore necessary so that the Board can evaluate the Respondent's technical ability and ethical awareness in managing psychotherapeutic relationships.

The Board has serious questions regarding this Respondent's judgment, self-control, character and emotional stability. Therefore, a complete psychological evaluation of Dr. Brewer is indicated. When Dr. Brewer subsequently appears before the Board, the Board must be able to be certain that Dr. Brewer will be able to function effectively as a psychologist.

IT IS THEREFORE THE ORDER of the Iowa Board of Psychology Examiners that license number 291 issued to Dr. John Douglas Brewer to practice as a psychologist is hereby suspended for a period of four (4) years. IT IS THE FURTHER ORDER of the Board that all of the period of suspension except one (1) year is stayed. IT IS THE FURTHER ORDER of the Iowa Board of Psychology Examiners that immediately upon issuance of this Decision, the Respondent's license is placed on probation and the Respondent shall immediately cease taking new patients. Within 30 days from the issuance of this Order the Respondent shall properly refer to other qualified mental health providers those patients whose treatment he has not completed. The purpose of this probationary term is to protect the public.

IT IS HEREBY THE FURTHER ORDER of the Board that the Respondent will take a total of two graduate courses in psychotherapy theory and technique. The courses must be three-semester-hour university courses and must be in sequence. In addition, IT IS THE FURTHER ORDER OF THE BOARD that the Respondent will take one three-semester-hour university graduate course in ethics. All courses must be approved by the Board. The Respondent's period of suspension will continue until the Respondent has successfully completed all required courses. Successful completion means the Respondent must receive a grade of "B" or better in the courses. The period of suspension will begin thirty (30) days from the date of the issuance of this Decision. The period of suspension will end once the Respondent provides proof to the Board that he has successfully completed the two courses in psychotherapy and the ethics course, but not sooner than the end of a one-year period. Costs of the re-education will be paid by the Respondent.

IT IS THE FURTHER ORDER of the Iowa Board of Psychology Examiners that after the Respondent has successfully completed the ethics course, he must take and successfully pass a written ethics examination. The Board will determine which ethics examination he will take and criteria for passage once he has successfully completed his ethics course. Costs of the testing will be paid by the Respondent.

IT IS THE FURTHER ORDER of the Board that after the Respondent has completed the two advanced psychotherapy courses and the ethics course, the Respondent must appear before the Board. At this appearance, the Respondent will discuss with the Board his technical abilities as a psychologist, including management of psychotherapeutic relationships. If at that time, the Board believes that further education is necessary, the Board may order further education, and the period of suspension which is not stayed may be extended by the Board. Costs of any further education, if ordered, will be paid by the Respondent.

IT IS THE FURTHER ORDER of the Iowa Board of Psychology Examiners that within ninety (90) days after the issuance of this Order the Respondent must be evaluated by a licensed psychologist. The Board must approve the choice of psychologist. The Respondent must complete the evaluation, and participate in any treatment recommended by the evaluator and approved by the Board. The Respondent must submit the necessary waivers, both with respect to the evaluator-patient relationship, and with respect to the Iowa Code 258A.6(1) confidentiality requirement, so that the Board and the evaluator may discuss the Respondent's evaluation and treatment based on the evidence presented at this hearing, and on the results of the evaluation. The Iowa Code section 258A.6(1) waiver will apply only to the Board members and the evaluator. The evaluator will be provided a copy of this Decision. The Respondent must sign a waiver so that the Board may have access to the evaluation, and may discuss its results

with the evaluator. The Respondent must sign a waiver of 258A.6(1) as to the Board and evaluator only, so that the Board will be able to discuss with the evaluator evidence which came out in this hearing. The Board and the evaluator will discuss the results of the evaluation, and will discuss appropriate treatment for the Respondent. The Board must approve terms of treatment. If the Board makes recommendations regarding treatment, the evaluator and the Respondent must follow the recommendations of the Board. Costs of the evaluation and treatment will be paid by the Respondent.

IT IS THE FURTHER ORDER of the Board that during the first period of suspension which is not stayed, the Respondent may have no patient contact whatsoever. After the Respondent's period of suspension is lifted, and the Respondent is given a limited license, the Respondent's limited license will be on probation for a period of two (2) years.

IT IS THE FURTHER ORDER of the Board that once the Respondent has provided evidence of successful completion of the advanced psychotherapy courses and the ethics course, has appeared before the Board, and does not require further education, the Respondent's license will be placed on probation subject to the following terms and conditions:

a. For the first six months of probation, the Respondent's practice is restricted to administration and scoring of psychological tests in a setting as described below in paragraph "b" and where the work that he does is supervised;

b. During the second six months of the second year, the Respondent may begin to see patients, but only in the following setting: 1) it cannot be in a private practice, and 2) it may be only in a community, county or state-owned mental health facility, and 3) the Respondent must submit his choice to the Board for approval. The Respondent's work must be supervised, and all of his work must be cosigned by his supervisor;

c. The Board must approve the Respondent's supervisor and the supervisor must report to the Board upon request of the Board. Furthermore, there will be a Board liaison assigned to monitor the Respondent's progress. The Respondent's supervisor and the Respondent must submit a plan to the Board specifying supervisory procedures during this probationary period. The Respondent and his supervisor must cooperate with the Board liaison. Before the Respondent may move into the next stage of his probation, he must present a plan to the Board with specifics regarding implementation of the next phase of probation for Board approval. Until he obtains Board approval, the Respondent may not go on to the next level of the probation. The Respondent may not see patients until the plan is submitted and approved by the Board;

d. During stage three of the Respondent's probation, the Respondent may work in the private sector. However, he must be supervised by a licensed psychologist for a one-year period of time. The licensed psychologist supervisor must be approved by the Board, and the Respondent must submit a plan of implementation for Board approval prior to actual work. This plan must include where the Respondent will work, and the Board must approve where the Respondent will work. The Respondent's practice is not limited, but he must be supervised by the licensed psychologist. The Respondent and the supervisor will cooperate with the Board liaison in the Board liaison's monitoring of the supervision. Upon successful completion of the period of probation, the Respondent's license will be fully restored.

IT IS THE FURTHER ORDER of the Board that during the periods of suspension and probation, the Respondent will file quarterly reports with the Board regarding his progress. If employed in the field of psychology, the Respondent will show this Decision and Order to every supervisor and employer, and will cause his supervisor and/or employer to submit quarterly progress reports to the Board. Reports showing less than adequate progress may be cause for further Board review.

IT IS THE FURTHER ORDER of the Iowa Board of Psychology Examiners that if the Respondent does not comply with all or part of the terms of this Decision, after notice and opportunity for hearing, the Board may impose part or all of the suspension which was stayed. This decision will be introduced as evidence at that hearing regarding lifting of the stay.

IT IS THE FURTHER ORDER of the Board that the Board will assign a person to monitor the conditions of this Decision. The Respondent must report to and must cooperate with that person. The frequency of reporting and related documentation will be determined by the monitor in consultation with the Board. Costs of such supervision will be paid by the Respondent.

Dated this 29<sup>th</sup> day of April, 1991.

Rick Jennings, Ph.D. by ACC  
Rick Jennings, Ph.D.  
Vice Chairperson of the Board

Amy Christensen Couch  
Amy Christensen Couch  
Administrative Law Judge  
Iowa Department of Inspections and Appeals

ACC/jmm

STATE OF IOWA  
BEFORE THE BOARD OF PSYCHOLOGY EXAMINERS

IN THE MATTER OF THE LICENSE  
TO PRACTICE AS A PSYCHOLOGIST  
OF:

\*

\*

DIA NO.

JOHN D. BREWER,  
Respondent.

\*

NOTICE OF HEARING AND  
STATEMENT OF CHARGES

You are hereby notified that a disciplinary hearing has been scheduled concerning your license to practice as a psychologist in the State of Iowa. The hearing will begin at 8:30 a.m., September 14, 1990, in the 1st Floor, Hearing Room 2 of the Lucas State Office Building, Des Moines, Iowa. You should be present at the hearing. Section 17A.12(3) of the Iowa Code provides if a party fails to appear at the hearing, the hearing may proceed and a decision made in the absence of the party. You may be represented by legal counsel, present evidence, and cross-examine witnesses. The authority for the hearing is Sections 258A.3 and 258A.6 of the Iowa Code and board rules 240.201 through 240.212.

The hearing will concern whether or not disciplinary action should be taken concerning your license to practice as a psychologist given the alleged violations:

1. By showing a personal interest which might impair one's professional judgement or increase the risk of client exploitation, you did see a client both inside and outside the professional office while the individual was

still a client actively involved in treatment and had a sexual encounter with the client on or about July 28 of 1989 in violation of board rules:

645 Iowa Admin. Code § 240.212(1)"c", "[k]nowingly making misleading, deceptive, untrue or fraudulent representations in the practice of a profession or engaging in unethical conduct or practice harmful or detrimental to the public. Proof of actual injury need not be established."

645 Iowa Admin. Code § 240.212(6), "[n]egligence by the licensee in the practice of the profession, which is a failure to exercise due care including negligent delegation to or supervision of employees or other individuals, whether or not injury results; or any conduct, practice or conditions which impair the ability to safely and skillfully practice the profession."

645 Iowa Admin. Code § 240.212(7), "[p]rohibited acts consisting of the following: subparagraph "f" "[a]ny sexual intimidation or sexual relationship between a psychologist and a client."

645 Iowa Admin. Code § 240.212(14)"a"(5), "[a]s practitioners, psychologist know that they bear a heavy social responsibility because their recommendations and professional actions may alter the lives of others. They are alert to personal, social, organizational, financial, or political situations or pressures that might lead to misuse of their influence,"

645 Iowa Admin. Code § 240.212(14)"b"(5), "[p]sychologists recognize that their effectiveness depends in part upon their ability to maintain effective interpersonal relations, and that aberrations on their part may interfere with their abilities. They refrain from undertaking any activity in which their personal problems are likely to lead to inadequate professional services or harm to a client; or, if engaged in such activity when they become aware of their personal problems, they seek competent professional assistance to determine whether they should suspend, terminate or limit the scope of their professional or scientific activities."

645 Iowa Admin. Code § 240.212(14)"f"(1), "[p]sychologists are continually cognizant of their own needs and of their inherently powerful position vis a vis clients, in order to avoid exploiting their trust and dependency. Psychologists make every effort to avoid dual relationships with clients or relationships which might impair their professional judgment or increase the risk of client exploitation. Examples of such dual relationships include treating employees, supervisors, close friends or relatives. Sexual intimacies with clients are unethical."

645 Iowa Admin. Code § 240.212(14)"f"(5), "[t]he psychologist attempts to terminate a clinical or consulting relationship when it is reasonably clear that the consumer is not benefitting from it."

2. By giving marijuana to a client to smoke in violation of board rule:

645 Iowa Admin. Code § 240.212(7)"d," "[o]btaining, possessing, or attempting to obtain or possess a controlled substance without lawful authority; or selling, prescribing, giving away, or administering controlled substances."

The hearing will be presided over by the Board which will be assisted by an administrative law judge assigned by the Iowa Department of Inspections and Appeals. The attorney who will represent the State of Iowa is Chris Odell, Assistant Attorney General, Office of the Attorney General, Hoover State Office Building, Des Moines, Iowa 50319, telephone (515)281-8330. Copies of all filings with the Board should be submitted to Ms. Odell.

Dated this 20th day of July, 1990

A handwritten signature in cursive script, reading "Carl Davis", written in dark ink.

Carl Davis, Ph.D.

Chairperson

Iowa Board of Psychology Examiners

## BEFORE THE IOWA BOARD OF PSYCHOLOGY EXAMINERS

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|                    |   |                                      |
|--------------------|---|--------------------------------------|
| IN THE MATTER OF : | ) | CASE NO. 96-020                      |
|                    | ) |                                      |
| JOHN BREWER, Ph.D. | ) | <b>COMBINED STATEMENT OF CHARGES</b> |
|                    | ) | <b>AND INFORMAL SETTLEMENT</b>       |
| Respondent.        | ) | <b>AGREEMENT</b>                     |

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COMES NOW the Iowa Board of Psychology Examiners (Board) and John Brewer, Ph.D. (Respondent) and pursuant to Iowa Code sections 17A.10(2) and 272C.3(4) enter into the following statement of charges and informal settlement agreement.

**STATEMENT OF CHARGES**

1. Respondent was issued license number 00291 to practice psychology in Iowa on September 1, 1978, and the Respondent's license is current.
2. On or about April 29, 1991, the Iowa Board of Psychology Examiners (Board) issued an order suspending the Respondent's license to practice and imposing other discipline upon the Respondent for violations of Iowa Code chapter 147 and certain administrative rules promulgated by the Board.
3. The Board has monitored the Respondent's compliance with the terms of its April 29, 1991 order.
4. Following the completion of the period of suspension imposed by the Board, the Respondent was ordered to complete a period of probation during which Respondent's practice was limited by certain terms and conditions.

5. Respondent's successful completion of the terms and conditions of probation was required as a condition precedent to the removal of the restrictions placed upon Respondent's license by the Board's 1991 order.

6. The Respondent has not completed all of the terms and conditions imposed by the Board's order of April 29, 1991.

7. During the pendency of the Board's order of April 29, 1991, the Board received new complaints regarding the Respondent.

8. The new complaints concerning the Respondent alleged violations of Iowa Code §147.55(3), §272C.3(2)(b), and the administrative rules promulgated at 645 IAC §§240.212(1)(c), (d) and 240.212(14).

9. The Board investigated the allegations of the complaints and found probable cause to initiate a disciplinary hearing based upon alleged violations of the Board's ethical and other standards governing the practice of psychology in Iowa.

10. This disciplinary hearing of charges, by the consent of the parties, is being filed simultaneously with the filing of an informal settlement agreement which shall constitute the final resolution of the disputed issues.

11. Accordingly, pursuant to applicable Iowa statutes and the Administrative Code rules and the Board is authorized to impose discipline against the Respondent.

#### **INFORMAL SETTLEMENT AGREEMENT**

12. The Board has jurisdiction over the parties and the subject matter.

13. By executing this informal settlement agreement, the Respondent expressly waives the right to notice and hearing on the charges.

14. Respondent is freely and voluntarily entering into this agreement and he agrees that the State's counsel may present this agreement to the Board.

15. This Statement of charges and Informal Settlement Agreement is subject to approval of the Board. If the Board fails to approve this Statement of Charges and Informal Settlement Agreement, it shall have no force or effect to either party.

16. The Board's approval of the Statement of Charges and Informal Settlement Agreement shall constitute the resolution of a contested case proceeding and shall be a FINAL ORDER of the Board.

17. The Respondent admits that he has not complied with all of the terms and conditions imposed by the Board's April 1991 order and, responds to the current allegations by stating that he denies the validity of such charges, but does not wish to contest them and desires, in lieu thereof, to voluntarily surrender his license to practice clinical psychology in the State of Iowa.

18. The Respondent agrees to voluntarily surrender his license to practice psychology in the State of Iowa and also agrees that, pursuant to Iowa Code Section 154B.8, a voluntary surrender must be accompanied by written statement of intention and when accepted by the director of public health, the Respondent agrees that the voluntary surrender shall have the same force and effect as an order of revocation issued by the Board.

19. The Respondent hereby agrees not to engage in the practice of psychology in the State of Iowa.

20. The Respondent agrees that should he ever seek reinstatement of his license to practice psychology that he shall be required to submit evidence to the Board's satisfaction that he is prepared to practice in a manner which shall conform the State statute and Administrative Code.

21. This Statement of Charges and Informal Settlement Agreement are public records available for inspection and copying in accordance with the requirements of Iowa Code Chapter 22 and shall become a part of the permanent record of Respondent.

  
\_\_\_\_\_  
John Brewer, Ph.D.  
Respondent

The statement of charges and informal settlement agreement is approved by the Board  
on this 12th day of September, 1997.

  
\_\_\_\_\_  
Dr. Robert Jones, Chair  
Iowa Board of Psychology Examiners

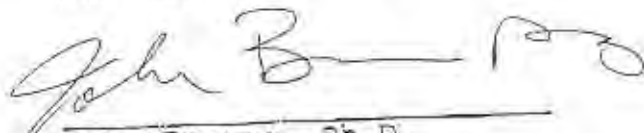
Mr. Christopher G. Atchison  
Director  
Iowa Department of Public Health  
Lucas State Office Building  
Des Moines, Iowa 50319

Dear Mr. Atchison:

You are hereby notified that in accordance with the provisions of Iowa Code section 154B.8, this letter shall constitute my written statement of intention to voluntarily surrender the enclosed license to practice psychology in the state of Iowa. You are further notified that I fully understand that upon your acceptance of this voluntary surrender of my license, such acceptance shall have the same force and effect as an order of revocation.

You are further notified that I hereby affirm my decision not to engage in the practice of psychology in the state of Iowa and I hereby absolve and release the Iowa Department of Public Health and the Iowa Board of Psychology Examiners from any further right, claim, or title to the certificate of license and to any of the rights, privileges and honors pertaining thereto.

This written statement is my intention to voluntarily surrender my psychology license is being signed by me on this 15 day of August, 1997, as my own voluntary act and deed.

  
John Brewer, Ph.D.  
Respondent

  
James E. Shipman  
Attorney for Respondent

\_\_\_\_\_  
Witness

ATTACHMENT A